

Not documented

MRN: 66863369 | Age: Not documented [Male]

Admission: 25-03-2026 | Discharge: Not documented

CHIEF COMPLAINT & HISTORY

The patient is a male who presented to the hospital for management of a gangrenous right little finger involving the middle and distal phalanges. The clinical history is significant for a right little finger crush injury accompanied by a compound fracture. Upon presentation, the patient required surgical intervention for reconstruction due to the presence of gangrenous tissue. The patient's functional baseline prior to admission required assistance with activities of daily living, including bathing, dressing, eating, walking, and toileting. There are no documented comorbidities such as hypertension or diabetes in the provided history, although a capillary blood glucose reading was noted during the admission period. No allergies were documented upon admission. Social history regarding smoking or alcohol use is not documented.

PHYSICAL EXAMINATION**VITAL SIGNS**

- Blood Pressure: 130/80 mmHg (Elevated)
- Heart Rate: 88 bpm (Normal)
- Respiratory Rate: 18 breaths/min (Normal)
- SpO2: 98% on room air (Normal)
- Temperature: 98.4 F (Normal)
- Pain Score: 2/10
- GCS: E4 V5 M_ (Total GCS 15/15)

GENERAL APPEARANCE

The patient was noted to be oriented during nursing assessments. Level of consciousness was stable with a GCS of 15/15.

SYSTEM EXAMINATION

- HEENT: Not documented.
- Cardiovascular: Pulse 88 bpm; rhythm and sounds not explicitly detailed.
- Respiratory: Respiratory rate 18/min; work of breathing not documented.
- Abdomen: Not documented.
- Musculoskeletal: Right little finger status post reconstruction; crush injury and compound fracture noted.
- Neurological: Patient is oriented; GCS 15/15.
- Skin: Gangrenous middle and distal phalanx of the right little finger noted upon admission; status post local flap procedure.

LABORATORY RESULTS

- Blood Glucose: GRBS 133 mg/dl (Interpretation: Diabetic)
- Hematology: Not documented.
- Chemistry: Not documented.

IMAGING/PROCEDURES

- Procedures: Local flap procedure performed under block. Specifically, a cross-finger flap detachment was performed, and the flap was inset using 4-0 Ethilon sutures.

RISK ASSESSMENT SCORES

- Fall Risk: Score 2 (Risk identified)
- Pressure Ulcer Risk (Braden): Score 23 (Low Risk)
- DVT Risk: Not assessed
- Aspiration Risk: Score 0 (Low Risk)

FUNCTIONAL STATUS

- ADLs: Patient requires assistance with bathing, dressing, eating, walking, and toilet use.
- Mobility: Ambulant, but requires assistance or supervision for mobility, transfers, or ambulation.
- Communication ability: Not documented.

ASSESSMENT

PRIMARY DIAGNOSIS

- Middle and distal phalanx gangrenous right little finger. This condition resulted from a crush injury and compound fracture of the right little finger, necessitating surgical reconstruction via local flap.

SECONDARY DIAGNOSES

- Right little finger crush injury with compound fracture.
- Anxiety (Nursing diagnosis identified during stay).

CLINICAL COURSE

The patient was admitted for reconstruction of the right little finger due to gangrene of the middle and distal phalanges. The patient underwent a surgical procedure involving a cross-finger flap detachment and flap inset using 4-0 Ethilon under regional block. During the hospital course, the patient was monitored for stability; vitals remained stable, including a blood pressure of 130/80 mmHg and a pulse of 88 bpm. Nursing assessments identified anxiety, for which psychological support was provided. The patient remained oriented with a GCS of 15/15. The patient is currently stable following the reconstructive procedure.

PROGNOSIS

The short-term prognosis is dependent on the successful integration of the cross-finger flap and the prevention of infection at the surgical site. Long-term considerations include the functional recovery of the right hand and management of the previous crush injury.

DISCHARGE DISPOSITION

The patient is being discharged to home. Due to the requirement for assistance with ADLs and mobility, home support or assistance will be necessary.

PLAN

MEDICATIONS

- Tab. Pyrigesic 650 mg, 650 mg, QD, Oral - For pain management.
- Tab. PAN 40 mg, 40 mg, OD, Oral - For gastric protection.

DIET

- Diet: NPM (Not documented/Normal)

ACTIVITY

- Activity: Ambulant with assistance/supervision required for mobility and transfers.

NURSING CARE

- Wound care: Monitor surgical site (right little finger flap) for signs of ischemia, infection, or hematoma.
- Fall precautions: Patient is identified as a fall risk; implement supervision during ambulation and transfers.
- Pressure ulcer prevention: Braden score 23 indicates low risk; continue standard repositioning.

PATIENT EDUCATION TOPICS COVERED

- Post-operative wound care and flap monitoring.
- Fall prevention strategies.
- Management of anxiety and psychological support.

FOLLOW-UP

- Not documented.

RED FLAGS

- Increasing pain at the surgical site not relieved by medication.
- Discoloration (blue, purple, or black) of the flap or finger.
- Excessive swelling or bleeding from the wound.
- Fever or chills.
- Foul-smelling discharge from the surgical site.
- Loss of sensation in the affected finger.

******* END OF RECORD *******